A3 Improvement Plan: Low Depression Scores

Torrance Memorial's doctors' network · Medicare quality scores for depression

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1. Why this matters

Medicare scores Torrance Memorial's doctors' network on quality. Better scores mean the network keeps more of the money it saves Medicare (it earned **$5.15 million** in 2024). Mental health care is also one of Torrance Memorial's community goals for 2025–2028.

2. Where we are now, and the goal REAL DATA, 2024

Measure	Torrance Memorial	Typical network
Depressed patients counted as recovered after a year	**7 in 100**	14 in 100
Patients checked for depression (and followed up)	**74 in 100**	84 in 100
Blood pressure under control	**91 in 100 (top 5%)**	80 in 100

The blood-pressure result shows the team can fix a gap. Depression is the next one to fix.

Goal: within one year, match the typical network nationally: **14 in 100** recovered and **84 in 100** checked.

3. Why it happens SIMULATED DATA

> **Patients may be getting better, but they aren't given the survey that proves it.**

There's a **short 2-question** depression survey and a **full 9-question** one. Only the full survey can show a patient recovered. Adults usually get only the short one; teens get the full one at every checkup.



Counted as recovered after a year.

Also: half of missed depression checks happened at **yearly checkups**, and young adults (18–34) are checked least because many skip checkups.

4. The fix: three small system changes

1. **Give the full survey a year later** to anyone who had a high score before.
2. **Require a depression check at every yearly checkup.**
3. **Add a box to record the score** at pregnancy visits.

In the simulation, these raise the share of patients checked from **71% to 80%**, even if only half of visits use them. They need no new staff.

5. Plan

When	What
Weeks 1–2	Check Torrance Memorial's real records: do adults mostly get the short survey?
Weeks 3–6	Build the three changes with IT. Try them at 1–2 clinics.
Months 2–3	Track results monthly. If they work, roll out to all clinics.
Months 3–6	Reach young adults: check for depression at any visit, not just checkups.

6. How we'll know it's working

- **Monthly:** share of checkups that include a depression check. It's been steady around 77% for three years. **A month above 80%** is the first sign it's working.
- **Monthly:** share of past high scorers who get the full survey a year later.
- **Every quarter:** the two Medicare depression scores.

Risks

- **Too many pop-up alerts** annoy staff, so alerts should only appear for patients who are due.
- **If adults already get the full survey,** the problem is the treatment itself, and the next step is more mental-health support.

Real data: Medicare Shared Savings Program results, 2024 (Torrance Memorial Integrated Physicians vs. all networks). **Simulated data:** fake patient records built to look real, because real records are private; results are a lead to confirm, not Torrance Memorial's actual numbers. Terms: full survey = PHQ-9; short survey = PHQ-2; recovered = remission (PHQ-9 below 5). Details: github.com/yeonancy56/depression-care-gap-analysis